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Name: نادية عبدالرؤف السيد سعود National ID: 26411180102564	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



MDT FORM

18 / 20

G

Name	نادية عبدالرحمن	MRN	18
Age	60 yrs	ID	
Menopausal status	post	Phone	01110264108
NRS SCORE		Marital status	
BMI		Family history	ve
CO morbidity	DM (controlled)	REASON OF MDT	

Clinical Data

BX:

ER:

8/8

PR:

8/8

HER 2:

+2

KI67:

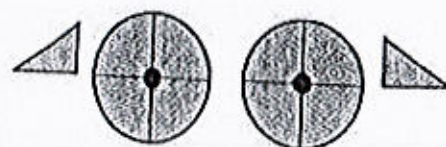
Sonomammography:

Mets Workup:

- CT CAP: Pending
- Bone Scan: PD

R

L



TNM

Recommendations:

MBG to bone since 2020
→ on Femora + Xeloda till 4/2024
Stopped Xeloda due to severe toxicity
on Femora only till 8/2023 → Bone PD

MDT OUTCOME & PLAN

① New CTCAP For addition of CDK4/6
② Shift to Fulvestrant + Palbocicli + Zoledronic acid

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name:	ناديه عبدالرؤف السيد سعود	
MR. :	8578	
Detailed Medical Report		
		السيد
تخية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 60-year-old individual with a known history of diabetes mellitus and right-sided hormone-receptor-positive breast cancer (ER 8/8, PR 8/8) with HER2 equivocal (+2) status. The disease progressed de novo to bone metastases in 2020. Initial systemic therapy consisted of letrozole combined with capecitabine until April 2024, when capecitabine was discontinued because of toxicity, followed by letrozole monotherapy until August 2025. Bone scan in August 2025 demonstrated progressive osseous disease with a new sacral lesion. Multidisciplinary discussion led to a change in regimen to fulvestrant plus palbociclib with zoledronic acid. A CT of chest/abdomen/pelvis in October 2025 showed no visceral metastases but multiple osteolytic bone lesions; a repeat CT in March 2026 revealed a right-breast mass with right axillary and mediastinal lymphadenopathy but no bony metastases. The plan is to initiate fulvestrant + palbociclib ± zoledronic acid (cycle 1), start IBRANCE (cycle 1, day 1), and administer Faslodex (cycle 1, day 15, pharmacy). The patient has now started cycle 1 of fulvestrant plus palbociclib and will receive the first dose of Faslodex on day 1 of cycle 1. The patient has a history of metastatic right-breast cancer to bone with ER 8/8, PR 8/8, HER2 equivocal status since 2020. Prior treatment included femara (letrozole) and capecitabine until August 2025, after which disease progression was noted on bone imaging. The multidisciplinary team considered Faslodex and IBRANCE, but approval for a new bone scan has not yet been obtained. The patient was diagnosed with metastatic breast cancer to bone in 2020 and received first-line Xeloda (capecitabine) plus femara (letrozole) until August 2025. The patient declined CDK 4/6 inhibitor therapy combined with Faslodex and therefore did not receive any systemic treatment after August 2025. A recent bone scan and CT chest/abdomen/pelvis both demonstrated stable disease, and the team is evaluating the possibility of initiating Faslodex as a single agent. The patient is scheduled to begin Faslodex as a single-agent therapy, with treatment commencing on cycle 1, day 1. The patient has now started cycle 1, day 1 of Faslodex single-agent therapy.		
تحريرا في ,		
	الوقت :	التاريخ : 20-09-2026
علياء احمد أبو النصر شهاب الدين		الطبيب المعالج :
يعتمد ,		

Patient Name : الأستاذ / نادية عبد الرؤف السيد	Request No. : 13004283
Patient No. : 26411180102564	Request Date : 31/12/2025
Path No. : 4060/25	Verification Date : 05/01/2026
Sex : Female	Financial Category : جوسف رومى - مبادره
Age : 61 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left axillary lesion ? nodal ? axillary fibroadenoma (BIRADS 4a) in follow up of pathologically proved right breast carcinoma.

Nature of Specimen: Tru-cut biopsy, collected on 31/12/2025, at 13:20 pm, cold ischemia time : 10 seconds.

Gross Examination:

- Three fragmented gray white tissue cores, the longest is 1 cm, totally submitted.

Microscopic Examination:

- Histopathological examination reveals cores of breast tissue showing a benign growth made up of proliferating small mammary ducts. Most of the ducts are compressed and closed, others are patent. The ducts are lined by inner columnar and outer myoepithelial cells. The stroma in between is formed of loose fibrous connective tissue showing marked hyalinosis. No lymphoid tissue. No evidence of malignancy in section examined.

Diagnosis:

Left axillary lesion, Tru-cut biopsy:

Consistent with hyalinized fibroadenoma. No lymphoid tissue. No malignancy.

Signature

Dr. Walaa Ibrahim Ghoneim.

Dr. Heba El-Shafeey.

Verified by :

Dr.Duaa Mohamed

Printing Date : 06/04/2026 12:02:19

Page 1 of 1

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name : Nadia Abdelraouf El-Sayed Saud

Date : 23-04-2026 11:41

Branch : Dokki

Medical No. : 358140

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : تعاقد مستشفيات

Tc-99m MDP ISOTOPIC BONE SCAN

61-year-old female patient. Known history of breast carcinoma; complains of bone aches. Evaluation for suspected skeletal metastases.

Technique: Whole-body planar bone scintigraphy was performed in anterior and posterior projections following appropriate uptake time.

Findings:

- Heterogeneous tracer uptake is noted within the lower dorsal vertebrae. This pattern is likely attributed to the patient's history of surgical intervention in this region.
- Intense focal tracer accumulation is identified in the left sacroiliac region, posterior end of the left 9th rib, mid-shaft of the right humerus (prominent lesion) as well as along the left clavicle.
- Expected age-related degenerative changes are visualized at multiple levels of the spine and around major joints (shoulders and knees) without demonstrating a suspicious metastatic pattern.

CLINICAL IMPRESSION:

- **Multifocal skeletal metastases:** Multiple osseous lesions at the aforementioned sites, highly suspicious for active metastatic disease.
- **Post-surgical changes:** Heterogeneous activity in lower dorsal spine, consistent with prior surgery (correlate clinically and radiologically).
- **Fracture risk assessment:** Radiological correlation for right humerus and left clavicle to evaluate pathological fracture risk.
- Follow-up would be recommended.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Patient Name : الأستاذ / نادية عبد الرؤف السيد
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Age : 61 Y
Referring Doc :

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Printing By : .marwa mohamed youss



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient's Name: NADIA ABD AL RAOUF AL SAYED
Date: 2026-03-15
Referred by: Prof.Dr. CT CAP IV
Hospital: Goustave Roussey International Egypt

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:

****No available previous imaging:**

CECT CHEST:

- Right breast soft tissue mass lesion measures 5.6 x 3.6 cm.
- Multiple enlarged right axillary lymph nodes at level I, largest one measures 3.4 x 2.3 cm.
- No cavitary lesions or bronchiectatic changes.
- Multiple enlarged mediastinal lymph nodes, largest one measures about 2 x 1.8 cm.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

CECT ABDOMEN AND PELVIS:

- The liver is average in size showing homogenous CT density with no enhancing focal lesions or biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows no dense stones.
- Normal CT appearance of the spleen, pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No free collection.

THE SCANNED SKELETON:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- *Right breast soft tissue mass lesion with multiple enlarged right axillary lymph nodes...Highly suspicious nodal metastases.*
- *Multiple enlarged mediastinal lymph nodes*
- *No definite hepatic lesions or pulmonary nodules.*

*Dr.Heba El Balsky.MD
Lecturer and Consultant of Diagnostic and Interventional Radiology,
NLI-Menoufia University*

Dr.Heba Elbalshy

Dr.Heba Elbalshy

و طبقاً لعدد البيانات المرسلة من الضمان الاجتماعي

والبحث تبين أن

السيد (ة) فاديه عبدالرؤف السيد بطاقة رقم 2641118010254

و طبقاً إلى خطاب من التأمينات الاجتماعية بفيد دهم الانتفاع
بمكبر من القنات غير المستفدة بالتأمين الصحي، هذا الخطاب دون أي مسؤولية على الهيئة
الضمانية لتأمينات الصحي حال خضوع المواطن لتقارير التأمين بين الصمى المشمول

تاريخ الطر: 27-8-2023

الموجودات الشخصية

اسلام عبد الله



مركز دعم المعاومات
جميع بيانات...

مستمل 977313



الخطاب ساري لمدة ثلاثة اشهر من تاريخ الاصدار الا في حالة تغير حالة المواطن من قبل الهيئة الثانوية لتأمينات الاجتماعية
وزارة التأمينات الاجتماعية او وزارة التربية و التعليم

الادارة المركزية لتكنولوجيا المعلومات و اتصالات و اتصالات الطر

Patient Name: Nadia Abdalrwaf Al Sayed Saod	Date : 12/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 61 Year(s)
Medical ID: 2223/2025	Pat. ID: 122122

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN, AND PELVIS: -**

Findings:

Reported case of right breast cancer , revealed

I Chest:-

- Right breast RLO mass 4 x3.6 cm inseparable from underlying muscle .
- Bilateral enlarged axillary lymph nodes , the largest at the left side measures 3.3 x2.4 cm , the largest on the right side measures 2.5 x1.5 cm
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Right thyroid nodule 18 x11 mm .

II Abdomen & pelvis:

- The liver is of enlarged in size with homogenous CT texture. With no focal lesions .
- calculi gall bladder fossa
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.

Patient Name: Nadia Abdalrwaf Al Sayed Saod	Date : 12/10/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 61 Year(s)
Medical ID: 2223/2025	Pat. ID: 122122

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- D9 vertebral collabs, with fixation of D5, D6 , D10 , and D11 with rods and screws
- D7 ,D8 ,D9 , left 9th rib and left iliac bone osteolytic lesions .

Opinion:

- No pulmonary , hepatic metastatic deposits .
- D7 ,D8 ,D9 , left 9th rib and left iliac bone osteolytic lesions with collapse and likely metastatic .

Dr. Reham Khedr M.D.

Signature Reham.Khader
17/10/2025 4:01 PM



Patient Name:	Nadia Abd El-Rauof El-sayed	Patient Gender:	Female
Patient Code:	Q45674	DATE:	5/8/2025:04:44 PM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

Clinical history

- ✍ 60-year- old female patient with history of metastatic right breast cancer, for follow up.

Technique

- ✍ Anterior and posterior whole body scans were acquired 2 hours post IV injection of 740 MBq of the tracer.

Findings

Compared to last bone scan dated 2 September 2023, the current study reveals:

- ✍ Progression of previously noted osseous lesions involving mid shaft of right humerus, dorsal vertebrae more prominent at the 9th and 11th, left 11th rib posteriorly and left sacro-iliac region.
- ✍ Stationary course of the previously noted osseous lesion at the left scapula and left 5th rib posteriorly.
- ✍ Newly developed osseous lesion is seen at the sacrum.
- ✍ The previously noted faint tracer uptake at the mid shaft of left humerus is currently not obvious.
- ✍ Still noted arthritic changes of the shoulders, knees and feet.

Opinion

Follow-up study demonstrates:

Almost progressive course of the previous metastatic osseous lesions with a newly developed one, as described.

Dr. Aalaa Solaiman

Dr. Nahla Dessoki, MD

Patient name: Nadia Abdelraof Elsayed.

Date: 18-1-2021

CONTRAST ENHANCED MULTISLICE CT SCAN OF THE AB.
& PELVIS

A case of MUO to the bone revealed:

- The liver is average in size with homogenous CT texture. No focal lesions or contour irregularities .
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen .
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- No pathologically enlarged pelvi-abdominal lymph nodes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra renals.
- No ascites.
- Unremarkable pelvic structures.
- **Heterogenous texture of the mid dorsal vertebre with , internal fixation with plates and screws, another expansile osteolytic bony lesion is noted at the left iliac bone.**
- Large right breast heterogenous mass lesion with pathologically enlarged axillary lymph nodes.

Dr. Hadeer Radwan, MSc

18/1/2021

Name : Nadia-Abdel raouf

Multislice post-contrast CT scan of the chest with
multiplanar reformatting revealed:

- Right breast upper outer quadrant well defined soft tissue lesion with internal areas of cystic degeneration , it measures about 4.3 x3.7 cm along its maximum axial dimensions. Few enlarged ipsilateral axillary lymph nodes are noted , the largest measures about 2.5 x2 cm along its maximum axial dimensions.
- No consolidation, cavitary lesions or sizable pulmonary nodules.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormalities.

Dr. Alaa Sayed , M.Sc.